6.21 Hepatic Encephalopathy

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family say he is confused and sleepy **Objective** • Flapping tremor (asterixis) • Does not know the time or place • Ammonia raised	**ACTUAL** **Acute confusion** related to accumulation of ammonia as evidenced by disorientation and flapping tremor.	Back to his usual orientation within 72 hours.	• Give lactulose as prescribed, aiming for 2–3 soft stools a day. • Give rifaximin if ordered. • No sedatives. • Look for triggers and treat them: GI bleeding, infection, constipation, electrolyte imbalance. • Check orientation every shift.	Oriented: **goal met**.
Subjective • Family: "He forgets to eat." **Objective** • Cannot feed himself • Attention poor	**ACTUAL** **Feeding self-care deficit** related to confusion as evidenced by inability to feed self.	Gets enough to eat.	• Help him at mealtimes. • Do not cut protein unless it is ordered. • Chart what he eats.	Intake adequate: **goal met**.
Subjective • *No specific complaint* **Objective** • Confused • Weak	**RISK** **Risk for adult falls** as evidenced by risk factors: confusion and weakness.	No falls.	• Fall precautions in place. • Bed low and call bell within reach. • Help him when he walks.	No falls: **goal met**.